

Name *

First Name

Last Name

Email *

example@example.com

Organization *Need a W9? [Download](#)**1. Which best describes your primary role? ***

Medical Oncologist

Advanced Practice Provider (NP/PA)

Pharmacist

Nurse / Nurse Navigator

Other

2. Which best describes your practice setting? *

Academic / teaching center

Community practice with research affiliation

Community practice without research affiliation

Hybrid academic/community

Other

3. How many years have you been in independent clinical practice (post-training)? *

0–5 years

6–10 years

11–20 years

More than 20 years

Prefer not to answer

4. Approximately how many patients with HER2-positive metastatic breast cancer do you actively

manage? *

≤5

6–10

11–20

21–40

>40

5. Approximately how many patients have you personally treated with trastuzumab deruxtecan (T-DXd) to date (any line/setting)? *

0 (have not prescribed yet)

1–5 patients

6–15 patients

More than 15 patients

Unsure

6. For de novo or relapsed HER2-positive metastatic breast cancer, which initial systemic regimen are you most likely to use today (outside a clinical trial)? *

Taxane + trastuzumab + pertuzumab (THP / CLEOPATRA-like)

Chemo-sparing endocrine therapy + anti-HER2 antibodies (for select triple-positive patients)

Clinical trial (whenever possible)

Trastuzumab deruxtecan (T-DXd; Enhertu) + pertuzumab

No single go-to; case-by-case

If you chose no single go-to; case-by-case, briefly describe:

7. In HR+/HER2+ (triple-positive) metastatic disease, which approach best reflects your typical plan today? *

Trastuzumab deruxtecan (T-DXd; Enhertu) + pertuzumab in 1L (endocrine therapy added per patient factors)

Induction with THP, maintenance trastuzumab/pertuzumab (HP) + endocrine therapy

Induction with THP, maintenance HP alone (endocrine therapy added later/variable)

Endocrine therapy + HP up front (no cytotoxic) in select low-burden patients

Varies substantially by disease tempo/burden and patient factors

If you chose Varies substantially by disease tempo/burden and patient factors, briefly describe:

8. Before this discussion, how familiar were you with DESTINY-Breast09 (DB-09) first-line data in HER2-positive metastatic disease? *

- Not familiar
- Aware of the study, but not familiar with the results
- Somewhat familiar with results
- Very familiar with results
- Participated in DB-09 or related trial(s)

9. Overall, how would you characterize the impact of the DB-09 discussion on your first-line HER2-positive metastatic treatment thinking? *

- Practice-changing
- Directionally influential (may change select decisions)
- Interesting but still preliminary
- Unlikely to change my practice
- Unsure / need more data

10. Since the approval of T-DXd + pertuzumab in the first-line HER2-positive metastatic setting, how has this changed your treatment approach? *

- It is now my preferred first-line regimen for most eligible patients
- I use it frequently but still select patients carefully
- I use it selectively in high-risk or high-burden disease
- I have used it rarely so far
- I have not yet incorporated it into practice

11. In your current practice, approximately what proportion of newly diagnosed HER2+ metastatic patients are receiving T-DXd + pertuzumab in the first-line setting? *

- <10%
- 10-25%
- 26-50%
- 51-75%
- >75%
- Too early to estimate

12. Which patient/disease factors would most strongly push you toward a T-DXd-based first-line strategy? (Select up to 3) *

- High disease burden and/or significant symptoms where rapid/deep response is needed
- Baseline CNS metastases or high concern for CNS progression
- HR- / HER2+ biology (more aggressive disease)

Early relapse after prior curative intent taxane-based therapy
Patient preference to avoid taxane-associated side effects/logistics
Other

13. Which factors would most strongly push you to prefer THP (or another non-T-DXd first-line approach) instead? (Select up to 3) *

Baseline pulmonary disease (e.g., ILD, significant COPD) or other ILD risk concerns
Monitoring logistics (ability to obtain and review CT imaging on a regular schedule)
Concerns about cumulative fatigue/nausea over multi-year continuous therapy
Patient preference to limit time on cytotoxic-intent therapy (induction & maintenance model)
Payer coverage / reimbursement uncertainty
Limited prior experience/comfort with T-DXd toxicity management
Other

14. If you start a patient on T-DXd + Pertuzumab in first line, which approach best reflects how you would plan to treat in routine practice (while awaiting additional data)? *

Continue T-DXd + pertuzumab until progression or unacceptable toxicity (trial-like approach)
Planned induction with T-DXd ($\pm$ pertuzumab) for a fixed number of cycles, switch to HP maintenance in responders
Start T-DXd alone (without pertuzumab) for select patients
Would primarily continue using THP until guidelines change and/or OS data mature
Unsure / would individualize widely

15. If/when additional maintenance strategies become options (e.g., HP + palbociclib in HR+ disease; HP + tucatinib), how do you anticipate integrating them for patients who start with a T-DXd-based first-line approach? *

Would not extrapolate; would wait for data specifically after T-DXd induction before using maintenance add-ons
Would consider switching to HP maintenance and then adding the relevant maintenance agent in select patients
Would prioritize THP induction specifically to align with existing maintenance-trial designs
Unsure / will defer to guidelines, access, and institutional pathways
Other

16. In HR+/HER2+ metastatic disease treated with a T-DXd-based first-line regimen, when would you introduce endocrine therapy (assuming no contraindications)? *

Routinely during T-DXd-based induction
Sometimes during induction (select patients only)
Typically during a maintenance phase (e.g., after switching to HP)
Rarely / would not combine endocrine therapy with T-DXd-based induction
Not applicable / I do not manage HR+/HER2+ disease

17. Which single issue do you anticipate being the biggest real-world barrier to broader first-line adoption of T-DXd + pertuzumab? *

Nausea/vomiting and supportive care needs
Fatigue affecting long-term tolerability
Myelosuppression / lab monitoring burden
Infusion capacity / operational logistics
Coverage/cost / prior authorization
No single biggest barrier
Other

18. How would you most likely monitor for ILD/pneumonitis in patients receiving T-DXd in the first-line metastatic setting? *

Baseline CT chest only; symptom-triggered imaging thereafter
Baseline + scheduled surveillance CT imaging approximately every 6 weeks
Baseline + scheduled surveillance CT imaging approximately every 9-12 weeks
Surveillance imaging frequency driven primarily by routine disease assessment scans (varies)
Local protocol differs / radiology-driven pathway
Unsure / not standardized

19. What antiemetic approach would you most likely use with T-DXd (cycle 1) in your practice? *

Three-drug prophylaxis (5-HT3 antagonist + NK1 antagonist + dexamethasone)
Three-drug prophylaxis + routine olanzapine
Two-drug prophylaxis (5-HT3 antagonist + dexamethasone)
As-needed antiemetics only (no routine prophylaxis)
Institutional pathway / order set
Unsure

20. For a patient on first-line T-DXd + Pertuzumab with deep response but chronic fatigue/nausea impacting quality of life, what is your most likely next step? *

Optimize supportive care first; continue at same dose/schedule
Dose reduce T-DXd (one dose level) and continue
Extend dosing interval (e.g., occasional delays/spacing) while monitoring closely
Stop pertuzumab but continue T-DXd
Switch to HP maintenance ($\pm$ endocrine therapy) and monitor
Planned treatment break, then reassess
Other

21. If a patient receives first-line T-DXd + Pertuzumab and later progresses, which next systemic class/regimen would you most often consider first (assuming standard options and no trial available)? *

Tucatinib + capecitabine + trastuzumab (especially if CNS progression)
THP or another taxane-based HER2 regimen (if not previously used)
HP + endocrine therapy + palbociclib (PATINA-like) (HR+ only)
T-DM1
Chemotherapy + trastuzumab (other)
Clinical trial enrollment preferred

22. What additional evidence would most increase your confidence to adopt a DB-09-like strategy across your first-line patient population broadly? (Select up to 3) *

Guidance on treatment duration and de-escalation/maintenance strategies (e.g., when/if to transition off T-DXd)inclusion

Mature overall survival (OS) benefit

Longer-term safety data (including ILD incidence and outcomes)

Real-world implementation data (monitoring workflows, discontinuation patterns)

Biomarker subgroup clarity (e.g., factors that predict exceptional benefit or higher risk)

Clearer sequencing data after first-line T-DXd

Other

23. Thinking about today's discussion, is there one specific change you anticipate making in your practice (patient selection, monitoring, sequencing, supportive care, etc.)? Any additional questions for the faculty?

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